

TENTATIVE RULING(S) FOR August 31, 2026

Department S37 – Judge Winston Keh

This court follows California Rules of Court, rule 3.1308(b) for tentative rulings. (See San Bernardino Superior Court Local Emergency Rule 8.) Tentative rulings for each law & motion will be posted on the internet (<https://www.sb-court.org>) by 3:00 p.m. on the court day immediately before the hearing.

You may appear in person at the hearing although remote appearance by CourtCall is preferred. (See www.sb-court.org/general-information/remote-access).

If you do not have Internet access or if you experience difficulty with the posted tentative ruling, you may obtain the tentative ruling by calling the department (S-37) at (909) 708-8707 or the Administrative Assistant (909) 708-8756, who prepared the ruling.

If you (or both parties) wish to submit on the Tentative, notify the other party and call the department by 4:00 pm the day before and your appearance may be excused unless the Court orders you to appear.

You must appear at the hearing if you are so directed by the court in the tentative ruling. Be prepared to address those issues set forth by the court in its ruling.

UNLESS OTHERWISE NOTED, THE PREVAILING PARTY IS TO GIVE NOTICE OF THE RULING.

CIVSB2437792

Gutierrez v. Veritas Health Services, LLC et al

TENTATIVE RULING(S):

Before the Court is the Kindred Defendants' demurrer to the SAC's first cause of action for dependent adult abuse and neglect pursuant to Code of Civil Procedure section 430.10, subdivision (e), for failure to state facts sufficient to constitute a cause of action. Plaintiffs oppose and Defendants reply.

Analysis

The Kindred Defendants argue that, although the SAC contains additional factual detail, it still alleges at most professional negligence rather than the reckless withholding of custodial care required under the Elder Abuse and Dependent Adult Civil Protection Act. They further contend

the SAC does not establish the substantial caretaking or custodial relationship required by *Winn v. Pioneer Medical Group, Inc.* (2016) 63 Cal.4th 148, 157–158 (*Winn*), and that the allegations concerning understaffing, regulatory violations, and corporate authorization or ratification remain conclusory.

Plaintiffs respond that the SAC cures the deficiencies identified in the Court's prior ruling by alleging Gutierrez's total dependence on the Kindred Defendants for basic needs, the specific care-plan requirements, repeated failures to provide nutrition, hygiene, repositioning, and other required care, and resulting malnutrition and other harm. Plaintiffs contend these allegations sufficiently plead a custodial relationship, statutory neglect, and recklessness.

Elder abuse is a statutory cause of action and must be specifically alleged. (*Carter v. Prime Healthcare Paradise Valley* (2011) 198 Cal.App.4th 396, 410 (*Carter*); *Covenant Care, Inc. v. Superior Court* (2004) 32 Cal.4th 771, 790 (*Covenant Care*).) To state a claim for neglect under the Elder Abuse and Dependent Adult Civil Protection Act, a plaintiff must allege neglect as defined by Welfare and Institutions Code section 15610.57 and facts showing the defendant acted with recklessness, oppression, fraud, or malice in committing the neglect. (Welf. & Inst. Code, § 15657.) "Neglect" includes the failure to assist in personal hygiene or provide food, the failure to provide medical care for physical and mental health needs, the failure to protect from health and safety hazards, and the failure to prevent malnutrition or dehydration. (Welf. & Inst. Code, § 15610.57, subd. (b)(1)–(4).)

Acts of simple professional negligence are insufficient. (*Delaney v. Baker* (1999) 20 Cal.4th 23, 32.) Recklessness requires more than inadvertence, incompetence, unskillfulness, or a failure to take precautions; it involves a conscious choice of a course of action with knowledge of the serious danger to others. (*Id.* at pp. 31–32.)

As explained in *Carter*, to survive demurrer, a plaintiff alleging neglect must plead facts showing the defendant: (1) had responsibility for meeting the elder or dependent adult's basic needs; (2) knew of conditions that made the elder or dependent adult unable to provide for those needs; (3) denied or withheld goods or services necessary to meet those needs, either with knowledge that injury was substantially certain or with conscious disregard of the high probability

of injury; and (4) thereby caused physical harm, pain, or mental suffering. (*Carter, supra*, 198 Cal.App.4th at pp. 406–407.) The facts constituting the neglect and establishing the causal link between the neglect and injury must be pled with particularity. (*Id.* at p. 407; *Covenant Care, supra*, 32 Cal.4th at p. 790.)

The Act applies to a health care provider only where the provider had a substantial caretaking or custodial relationship with the elder or dependent adult involving ongoing responsibility for one or more basic needs. (*Winn, supra*, 63 Cal.4th at pp. 152, 157–158.) It is the nature of the relationship, rather than the defendant's professional status, that determines whether the requisite custodial relationship exists. (*Id.* at p. 152.)

1. The SAC adequately alleges a substantial caretaking or custodial relationship

The Kindred Defendants first argue that the SAC still concerns treatment provided by an acute care hospital to a critically ill patient and therefore does not establish the substantial caretaking relationship required by *Winn*.

The SAC alleges that Gutierrez resided at KH Ontario 24 hours per day and was “totally dependent” on its staff for his basic needs. He allegedly could not feed himself, reposition himself, bathe or dress himself, protect his own skin, or maintain his airway without assistance. The Kindred Defendants allegedly assessed Gutierrez and documented impaired bed mobility, feeding self-care deficits, bathing deficits, dressing and grooming deficits, and ineffective airway clearance. (SAC ¶¶ 20, 41–44.)

The SAC further alleges that KH Ontario undertook responsibility for feeding Gutierrez at each meal, bathing and washing him, dressing and grooming him, turning and repositioning him every two hours, providing toileting and incontinence care and changing soiled linens, inspecting his skin, and maintaining specified skin protection protocols. (SAC ¶¶ 20–21, 43.)

Those alleged responsibilities include basic custodial needs distinct from the provision of professional medical treatment. Feeding, bathing, dressing, grooming, toileting, and repositioning are needs an able-bodied and fully competent adult ordinarily would be capable of managing without assistance. They involve acts for which the failure to assist or provide constitutes neglect under the Act. (Welf. & Inst. Code, § 15610.57, subd. (b)(1)–(4).) The

allegations therefore support a reasonable inference that KH Ontario assumed ongoing responsibility for Gutierrez's basic needs within the meaning of *Winn*.

Defendants rely on *Winn*, *Oroville Hospital v. Superior Court* (2022) 74 Cal.App.5th 382 (*Oroville Hospital*), and *Kruthanooch v. Glendale Adventist Medical Center* (2022) 83 Cal.App.5th 1109 (*Kruthanooch*) to argue that the services alleged in the SAC remained professional medical and nursing care rather than custodial care. Plaintiffs distinguish those authorities and rely on *Stewart v. Superior Court* (2017) 16 Cal.App.5th 87 (*Stewart*) as involving circumstances more analogous to those alleged here.

The circumstances alleged here materially differ from *Winn*, where the decedent received outpatient medical care and returned home between appointments. (*Winn*, *supra*, 63 Cal.4th at pp. 152–153, 165.) They also differ from *Oroville Hospital*, which involved intermittent in-home wound care rather than responsibility for the patient's activities of daily living. (*Oroville Hospital*, *supra*, 74 Cal.App.5th at p. 405.) In *Kruthanooch*, the Court declined to hold that hospitalization alone created a custodial relationship, but recognized that a patient-provider relationship may expand into one in which the provider attends to the patient's most basic needs. (*Kruthanooch*, *supra*, 83 Cal.App.5th at pp. 1131–1133.)

Stewart is more analogous. There, the Court of Appeal concluded that an acute care hospital had assumed responsibility for assisting an inpatient with basic activities the patient could not perform independently. (*Stewart*, *supra*, 16 Cal.App.5th at pp. 102–103.) Here, the SAC similarly alleges that Gutierrez was unable to feed, bathe, dress, toilet, or reposition himself and depended on KH Ontario staff to meet those needs. (SAC ¶¶ 20, 42–43.)

Defendants argue that many of the other alleged activities, including airway suctioning, medication reconciliation, oxygen monitoring, and other ICU interventions, are professional medical or nursing functions requiring specialized judgment. But the existence of professional medical care does not eliminate the separately alleged responsibility for Gutierrez's basic custodial needs. The governing inquiry concerns the nature of the particular relationship and responsibilities assumed. (*Winn*, *supra*, 63 Cal.4th at p. 152.)

Accordingly, the SAC adequately alleges the requisite substantial caretaking or custodial relationship.

2. The SAC adequately alleges statutory neglect rather than merely deficient performance of medical services

Defendants next argue that Plaintiffs have simply supplied greater detail concerning professional negligence. That characterization does not account for all of the amended allegations.

The SAC alleges that Gutierrez could not feed himself; staff repeatedly left meal trays “within sight but out of reach” despite that limitation; staff failed to accurately monitor his intake and failed to ensure prescribed nutritional supplements were provided; he lost approximately 20 pounds in five days; and he became “acutely malnourished.” (SAC ¶¶ 22–23, 46.)

Those allegations fall within the statutory examples of neglect consisting of the failure to provide food and the failure to prevent malnutrition. (Welf. & Inst. Code, § 15610.57, subd. (b)(1), (4).) *Delaney* specifically recognizes that the omission of sufficient nutrition can constitute neglect rather than merely professional negligence. (*Delaney, supra*, 20 Cal.4th at pp. 34–35.)

The same is true of the hygiene allegations. The SAC alleges Gutierrez could not bathe or toilet himself and that his care plan required bathing, skin cleansing, linen changes following incontinence, and protective skin measures. Plaintiffs allege those services repeatedly were not provided, leaving Gutierrez with “soiled linens” and “dried stool” and contributing to worsening skin breakdown. (SAC ¶¶ 28–29, 43, 45, 50.) These allegations fit directly within section 15610.57, subdivision (b)(1)’s reference to the failure to assist in personal hygiene and, potentially, subdivision (b)(3)’s failure to protect from health and safety hazards.

The SAC also alleges specific care plan requirements, including feeding at each meal, two hour repositioning, heel offloading, incontinence care, skin checks each shift, and protective barrier applications, and identifies the measures Defendants allegedly failed to provide. (SAC ¶¶ 20–29, 43, 45–50.) In *Sababin v. Superior Court* (2006) 144 Cal.App.4th 81 (*Sababin*), the Court explained that where a facility knows it must provide certain care regularly but provides that care

only sporadically, withholding of care may have occurred. (*Id.* at p. 90.) Here, the SAC alleges repeated failures to provide multiple categories of required care identified in Gutierrez's care plan.

Defendants argue that an omitted nursing intervention does not automatically constitute dependent adult neglect. But the SAC, read as a whole, alleges more than an isolated omission or a medical procedure performed negligently. It alleges repeated failures to provide food, bathing, toileting and incontinence care, linen changes, turning and repositioning, and other basic services to a patient who allegedly could perform none of those functions himself.

Accordingly, the allegations concerning the withholding of basic custodial care are sufficient at the pleading stage to allege statutory neglect. The Court therefore need not determine on demurrer whether each additional allegation concerning medication reconciliation, respiratory treatment, or medical monitoring independently constitutes neglect under the Act.

3. The SAC adequately alleges recklessness at the demurrer stage

The closer issue is whether the SAC alleges more than negligence and adequately pleads the heightened culpability required by section 15657.

Defendants contend Plaintiffs must allege an intentional refusal to provide food, repositioning, or other care and argue that a collection of missed interventions cannot establish recklessness without allegations tying them to a common conscious decision. The law, however, does not require an intent to injure or an express decision to deny care. Recklessness may be shown by a conscious disregard of a known high probability of injury. (*Delaney, supra*, 20 Cal.4th at pp. 31–32.)

The SAC alleges facts permitting that inference at the pleading stage. The Kindred Defendants allegedly themselves assessed Gutierrez as unable to feed, reposition, bathe, dress, toilet, or otherwise care for himself and developed care plans identifying the particular assistance required and the frequency with which it was to be provided. (SAC ¶¶ 20–21, 42–43.) Despite that alleged knowledge, the SAC alleges repeated failures to provide several categories of required care over the course of Gutierrez's nine day admission. (SAC ¶¶ 23–29, 45–50.)

The allegations also include circumstances from which the seriousness of the risks allegedly was apparent. Gutierrez allegedly lost approximately 20 pounds in five days while unable to feed himself and nevertheless was not re-weighed or timely assessed by a dietitian. (SAC ¶¶ 22, 46.) His oxygen saturation allegedly fell to approximately 60 percent, notwithstanding a facility policy identifying 85 percent as a threshold requiring immediate physician notification, yet a physician allegedly was not timely notified. (SAC ¶¶ 24, 47.) ICU vital signs allegedly went undocumented for four hours notwithstanding a two-hour requirement. (SAC ¶¶ 26, 48.) Medication reconciliation allegedly remained incomplete for approximately seven days after admission. (SAC ¶¶ 27, 49.) Plaintiffs further allege DPH cited KH Ontario concerning these failures and identified resulting risks to Gutierrez's health and safety. (SAC ¶¶ 22, 24, 26–27, 46–49.)

The DPH allegations do not themselves establish recklessness; nor does a policy or regulatory violation, standing alone, transform negligence into dependent adult abuse. Those allegations are nevertheless relevant to the knowledge and risk alleged when considered together with the individualized care plans and repeated failure to provide basic care.

Sababin held that a significant pattern of withholding required care may permit a factfinder to conclude that the conduct resulted from choice or deliberate indifference. (*Sababin*, *supra*, 144 Cal.App.4th at p. 90.) That principle supports Plaintiffs' theory here because the SAC alleges repeated failures to provide multiple categories of care that the Kindred Defendants allegedly knew Gutierrez required.

Plaintiffs also rely on *Fenimore v. Regents of University of California* (2016) 245 Cal.App.4th 1339 (*Fenimore*), arguing that the alleged policy violations and understaffing further support an inference of recklessness. But unlike *Fenimore*, the SAC does not identify a staffing ratio or regulation knowingly violated, a particular staffing directive, or facts tying the alleged omissions in Gutierrez's care to a deliberate institutional staffing policy. (SAC ¶ 51.) The generalized allegation of a "conscious decision" to understaff KH Ontario therefore does not materially advance the recklessness analysis.

That deficiency is not dispositive because understaffing is not an element of the claim. Independently of the staffing allegations, the SAC alleges that the Kindred Defendants knew from their own assessments and care plans that Gutierrez could not meet his basic needs, identified the specific care he required, and nevertheless repeatedly failed to provide multiple categories of that care. Taken together, those allegations are sufficient at the pleading stage to support an inference of conscious disregard.

4. Defendants' challenge to corporate authorization or ratification does not warrant sustaining the demurrer

Defendants additionally argue that Plaintiffs have not adequately alleged authorization or ratification by an officer, director, or managing agent as required by Welfare and Institutions Code section 15657 and Civil Code section 3294, subdivision (b).

The SAC alleges that the Kindred Defendants' officers, directors, and managing agents knew of Gutierrez's significant weight loss, low oxygen saturation, missed vital sign monitoring, and delayed medication reconciliation, yet failed to take timely corrective action. (SAC ¶ 52.) It further alleges that KHO controlled KH Ontario's budget and participated in decisions concerning staffing and operations. (SAC ¶ 9.) Defendants contend these allegations are conclusory because Plaintiffs do not identify a particular officer, director, or managing agent or allege when and how such person acquired the asserted knowledge.

Defendants also correctly observe that the allegation that, after Gutierrez's death, "no further actions could be taken" for him because he had expired does not itself show that the Kindred Defendants approved or adopted the preceding conduct. (SAC ¶ 52.)

Nevertheless, this does not provide a basis to sustain the demurrer to the entire first cause of action. Welfare and Institutions Code section 15657, subdivision (c), requires satisfaction of the standards in Civil Code section 3294, subdivision (b), before the damages or attorney's fees authorized by section 15657 may be imposed against an employer. Thus, even assuming the SAC's allegations of corporate authorization or ratification are not sufficiently particularized to

support those enhanced remedies, that deficiency does not negate the otherwise sufficiently alleged statutory neglect and recklessness discussed above.

Moreover, a demurrer generally does not lie to only a portion of a cause of action. (*See Kong v. City of Hawaiian Gardens Redevelopment Agency* (2002) 108 Cal.App.4th 1028, 1047

[discussing that Court cannot sustain demurrer to part of a cause of action].) Accordingly,

Defendants' challenge to the corporate authorization and ratification allegations does not warrant sustaining the demurrer to the first cause of action.

RULING

For the reasons stated above, the Court:

OVERRULES Defendants THC – Orange County, LLC dba Kindred Hospital Ontario and Kindred Healthcare Operating, LLC's demurrer to the first cause of action for dependent adult abuse and neglect.